

HealthPlus — Red-Flag Escalation Table

Draft. Written by a non-clinician. Not adopted, not in use, and not to be relied on.

This is the safety layer of a medical and wellness travel platform for patients in India: six severity levels, the characterisation of what triggers each, what the user is shown, and whether a clinician sees it before the user does. It governs every response the system can produce.

It was drafted by me, and I am not a clinician. Charter §0.6 leaves the whole of this section unadopted until a Registered Medical Practitioner signs or corrects it, and the system currently refuses to answer health questions at all rather than run on an unsigned rule set. That is deliberate. It is also why nothing has launched.

The five questions on the last page are the actual ask. I would rather be corrected than agreed with.

1. The six levels

Lvl	Severity	Trigger characterisation	What the user gets	Clinician in loop before display
0	NORMAL	No red-flag pattern. Routine information, planning, logistics.	Normal response. No safety interstitial.	No
1	MONITOR	Non-acute symptom or risk factor worth tracking; mild, chronic, stable, or expected post-procedure course.	Normal response plus a persistent "watch for" panel naming specific worsening signs and the threshold at which to seek care. No alarm language.	No
2	WARNING	Could indicate a significant problem but is not time-critical — persistent unexplained symptom, medication-interaction concern, wound appearance change, post-op symptom outside expected course.	Response leads with a safety block. Concrete timeframe. Commercial content suppressed.	Yes , for any comparison content in the same response — not for the safety block itself
3	URGENT	Assessment needed within hours to about 72h — fever with post-operative wound signs, new focal neurological symptom without stroke pattern, moderate uncontrolled bleeding, suspected DVT, acute severe pain.	Interstitial takeover. Free-text conversation suspended. Pre-approved template immediately, with local emergency and urgent-care routing. No differential, no reassurance, no cost or provider content.	No — shown immediately. Generative output on the topic is blocked entirely rather than reviewed
4	CRITICAL	Serious, rapidly evolving, needs same-day emergency assessment — suspected sepsis, chest pain with cardiac features, severe dyspnoea, suspected post-operative haemorrhage, anaphylaxis history with current exposure, suicidal ideation without immediate intent or plan.	Hard takeover. Seek emergency care now, how to get there, what to bring, what not to do. Local emergency number and nearest ED from a maintained reference table. Crisis-line routing for mental-health triggers.	No — never gated

Lvl	Severity	Trigger characterisation	What the user gets	Clinician in loop before display
5	EMERGENCY	Immediate life-threat — stroke (FAST) signs, suspected MI, unresponsiveness, airway compromise, uncontrolled major bleeding, suicidal intent with plan or means, active overdose or poisoning.	Absolute takeover, single screen. “Call [emergency number] now.” Nothing above it. Minimal, non-negotiable content. All other functionality suppressed for the session.	No — never gated. Display latency is the safety metric

2. The counter-intuitive column, and why it is that way

At low severity, review is unnecessary. At **medium** severity review is *required* — because the system is producing individualised, non-obvious content that could be wrong. At **high** severity review is *forbidden as a gate* — because the correct action is already known, pre-approved and time-critical, and a queue would kill people.

So at URGENT and above the system stops being clever and gets out of the way. **Escalation replaces generation; it does not decorate it.** If you think that trade is wrong, say so — it is the single most load-bearing structural decision in the whole design.

3. Proposed time-to-care windows — the ones most likely to be wrong

Level	Stated timeframe
MONITOR	“If this changes or does not improve, arrange a routine appointment.”
WARNING	“Arrange to be seen within the next few days.”
URGENT	“You need to be assessed today / within 24 hours.”
CRITICAL	“Go to an emergency department now.”
EMERGENCY	“Call [emergency number] immediately.”

Vague phrasing — “soon”, “if you feel it’s necessary”, “you may want to consider” — is prohibited at WARNING and above.

4. The medical-tourism triggers, which have no textbook

These are the ones with no established literature to copy, and where a generalist’s judgement is doing all of the work.

- **Post-operative patient who has flown home.** Continuity of care is broken and the operating team is in another country. Any post-op symptom is currently assessed **one level higher** than the same symptom in a non-travelling patient.
- **Patient currently abroad.** Emergency routing uses the destination country’s number, determined from stated location and never from the account address.
- **Pre-travel symptom.** New symptoms before departure are at or above WARNING, with “do not travel until assessed” where the pattern warrants — a triage instruction, not a fitness-to-fly determination, which the platform is forbidden to make.
- **Post-flight leg swelling, calf pain or dyspnoea in a post-operative patient** — currently at or above URGENT on a DVT/PE pattern.

- **Running out of, or being unable to import, a critical medication** — anticoagulant, immunosuppressant, insulin, antiepileptic — currently at or above URGENT.

5. High-risk profiles that raise the floor by one level

Age under 18 or 75 and over · pregnancy · immunosuppression · active malignancy · anticoagulation · transplant recipient · under 30 days post-operative · dialysis · known severe allergy or anaphylaxis history.

Set only from stated or provider-supplied data, never inferred.

6. The five questions — this is the actual ask

1. Are six levels right, or are the original four (Normal / Warning / Urgent / Emergency) better? **MONITOR and CRITICAL are additions by a non-clinician.**
2. Are the §3 windows clinically defensible in an Indian access-to-care context, where “be seen within a few days” may not be available?
3. Is “post-op symptom in a returned traveller = one level higher” the right rule, or too blunt?
4. Does raising the floor by one level for the §5 profiles over-trigger or under-trigger?
5. What is missing entirely?

Extracted from the HealthPlus Evidence & Safety Charter v1.0 §4. Charter §0.6 governs: none of this is adopted. Every severity, trigger and window above is a structural placeholder awaiting the clinical signature this document exists to obtain.